

## EUCAST guidance on use of fosfomycin i.v. breakpoints

Fosfomycin i.v. is used in monotherapy in infections, primarily urinary tract infections caused by *E. coli*. There are clinical data to support a breakpoint for *E. coli* in infections originating from the urinary tract, but it is currently not known whether this breakpoint will be predictive of efficacy in other infection types. The currently revised breakpoint of fosfomycin applies only to *E. coli* in infections originating from the urinary tract and is 8 mg/L, which includes the entire wild type population. The breakpoint is supported by PK/PD and clinical data. No breakpoints have been determined for other species, as most of them are inherently less susceptible and so have higher ECOFFs, have preclinical PK/PD targets which are not attained, and because there is limited clinical data to support the use of the agent in monotherapy. *P. mirabilis* is an exception from the general rule, since it has an ECOFF of 8 mg/L, but still there are no clinical data to support a breakpoint for this species.

Additionally, fosfomycin i.v. has also been used in combination therapy against several pathogens in various infections. It has not been possible to assess the added value of fosfomycin in combination therapy so far, and available data show no evidence to suggest that MICs are predictive of clinical efficacy. Thus, EUCAST will not issue guidance on how susceptibility testing would add value in the planning of combination therapy.

Fosfomycin ECOFFs can be found at [mic.eucast.org](https://mic.eucast.org).

Fosfomycin Rationale Document can be found at [Fosfomycin IV 1.0](#) (2023-11-23)

### References

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3. Ning Y, Trampuz A and Karbysheva S Clinical experience with intravenous (IV) fosfomycin for treatment of complex bone and implant-associated infections: prospective cohort, Poster ESCMID Global 2024, Barcelona, Spain, 2024, P3191
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